



Mental Health Case Management (Mental Health) Policy

Preamble

Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to¹:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all client/consumer care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Guidelines to Writing for Clinical Policy](#) for further information about Scope of Practice.

Background

- All clients/consumers will be allocated a Case Manager from entrance through to discharge from RKPG Mental Health (MH) service. This policy details the key principles of effective case management and describes the roles and responsibilities of performing case management within RKPG MH Services.

Introduction

- This policy is to be read in conjunction with:
 - * [RKPG Care of the Person Who May Be Suicidal \(Acute\) Policy](#)
 - * [RKPG Intake Meeting, Case Allocation and Case Review \(Mental Health\) Policy](#)
 - * [RKPG Mental Health Standardised Clinical Documentation and Process Standard Operating Procedure](#)
 - * [SMHS Care coordination in mental health](#)

Definitions

Risk assessment	A gathering of information and analysis of the potential outcomes of identified behaviours. Identifying specific risk factors of relevance to an individual, and the context in which they may occur. This process requires linking historical information to current circumstances, to anticipate possible future change.
Clinician	A health professional such as a medical officer (MO), nurse, social worker, occupational therapist, or clinical psychologist.

RESPONSIBILITY				NATIONAL STANDARDS
First Issued	Sep 2024	Revisions	NA	National Standards 1, 2, 5, 6
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Responsibility of	Co-Directors Mental Health	Endorsed by	DONM/DCS	
Title	Mental Health Case Management (Mental Health) Policy			

Case Manager	A mental health professional employed by a mental health service to help the consumer attain their recovery goals. Case Manager undertakes assessment, monitoring, planning, advocacy and linking of the consumers relevant services.
Case management	Case management is a process to meet a consumer's needs and promote recovery by placing the responsibility of coordinating assessment, treatment planning and delivery, as well as the brokering of additional support and specialist health services. The Case Manager works within a multidisciplinary team (MDT) and in collaboration with the consumer, family members, personal and professional carers, nominated person/s and with consent undertakes regular communication with all people involved.
Community Mental Health Service	A service that provides mental health assessment, treatment, and care for people in the community.
Client/consumer	A person who is currently using or has previously used a mental health service.
Multidisciplinary team (MDT)	A group of health professional who are members of different disciplines (psychiatrist, nurse, social worker etc.) each providing specific services to the consumer.
Digital Medical Record (DMR)	The Digital Medical Record (DMR) provides a digital version of the traditional paper based medical record. The DMR is the document where all health care providers contributing to the care of the client/consumer will record all relevant details of that care.
PSOLIS	Psychiatric Online Service Information System (PSOLIS) is the patient database used by WA public mental health services.
WebPSOLIS	WebPSOLIS is a mental health program to deliver the Statewide Standardised Clinical Documentation (SSCD) suite of documents to mental health staff/PSOLIS users.

Case management

- Case management includes the therapeutic engagement of a client/consumer in their mental health treatment. Therapeutic engagement is the active process of developing rapport for the purposeful gathering of information to assess mental state and risk, inform clinical decision-making, and promote safe therapeutic care. This relationship enables the Case Manager to monitor clinical presentation based on their knowledge and experience of the client/consumer.
- Therapeutic engagement and relationships play an important role in helping to create a psychologically safe culture and facilitates an environment where clinicians and case managed clients/consumers can speak up and collaborate in their mental health treatment.

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- Clinician safety is of utmost importance when attending community and face to face appointments, all efforts must be made to ensure clinician safety. If an unacceptable level of risk remains, then alternatives should be considered such as developing a safety plan, offering a taxi voucher and/or using an alternative venue.
- Clinicians may also wish to discuss with the client/consumer attending appointments with a support person, such as their carer/family member/next of kin (NOK)/nominated person, to enhance their comfort level in engaging with the service.
- The practice principles of case management are:
 - * Person centred care.
 - * Evidence-based.
 - * Trauma informed.
 - * Recovery focused and strength based.
 - * Socially inclusive and supportive of culture and diversity.
 - * Collaborative partnering with all stakeholders.
 - * Aligns with organisational governance and accountability, and within scope of practice.

Roles and responsibilities

- The case manager is the allocated person within the multidisciplinary team (MDT) who has the responsibility for coordinating, facilitating, and integrating mental health treatment, care, and support to a mental health carer/family member/NOK/nominated person.
- The allocated Case Manager will be:
 - * Any member of the MDT.
 - * Is competent in delivering evidenced based mental health care with a clinical skill set required to deliver the identified therapeutic and recovery focussed goals.
 - * Has a good understanding of mental disorders and their management.
 - * Can develop and maintain collaborative partnerships, including therapeutic relationships, with client/consumers, family members, carers, and significant others.
- It is a Case Managers responsibility to ensure:
 - * Clients/consumers they case manage have a full mental state assessment and risk assessment.
 - * face to face in line with the client/consumers Treatment Plan, Phase of Care, MDT and team Model of Care.
 - * Develop, review and update mental health treatment plan on WebPSOLIS.
 - * Develop a client/consumer focused safety plan.
- Ensure there is 3 monthly communication with the General Practitioner (GP), particularly with respect to physical health care, determining shared care needs and discharge planning.
- Case management continues across periods of inpatient mental health care, with the Case Manager providing in-reach, participating in family meetings and contributing to discharge planning.
- A nominated member of the MDT will be allocated as a contact during periods of absence of the primary Case Manager.

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Face to face contact

- Face to face meetings allow clinicians to read body language and non-verbal communication. This makes communication effective and avoids misunderstandings.
- Case Managers face to face contact will be determined by a mental state assessment, risk assessment, psychosocial assessment, treatment plan, current phase of care, team Model of Care (MoC) and MDT. At a minimum Case Managers will meet with client/consumers once every month. Any exceptions outside of this are to be reviewed and endorsed by the MDT.
- Medically managed clients, where the client's mental state has been assessed as consistently stable, determined by a mental state assessment, risk assessment, psychosocial assessment, phase of care, treatment plan, Model of Care (MoC) and MDT, must ensure face-to-face contact occurs at least once every 3 months.
- All efforts should be made on a regular basis to engage and maintain contact with clients in the service. This may include actions such as phone calls, electronic messaging, letters, and liaison with the referrer and other agencies that may be involved with the client's treatment and with whom the consent to contact has been obtained.
- If a Case Manager is unable to make contact with a client/consumer and there is no recorded consent to contact carer/family member/NOK/nominated person, the Case Manager is to discuss contacting the NOK with the MDT or if urgent the Team Leader/Coordinator/Consultant Psychiatrist.
- All staff to maintain awareness of, and sensitivity to cultural and language barriers which may hinder engagement. Clinicians must make every effort to access cultural consultation to discuss barriers and possible options for overcoming any identified cultural or language barriers.
- If a client/consumer declines to meet face to face with the Case Manager this is to be discussed at the MDT and the recommendation/outcome of the discussion recorded in the DMR.

Compliance monitoring

Compliance with this policy will be monitored by several methods: auditing the MDT checklist (RG180.1), Clinical Documentation Audit and reviewing live 'journey board' data.

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References

1. Australian Commission on Safety and Quality in Health Care (ACSQH). The National Safety and Quality Health Service Standards Safety and Quality Improvement Guide: Standard 1 Governance for Safety and Quality in Health Service Organisations. 2nd ed. (Version 2) 2021. Available from <https://www.safetyandquality.gov.au/our-work/assessment-to-the-nsqhs-standards/>

Links

[RKPG Care of the Person Who May Be Suicidal \(Acute\) Policy](#)

[RKPH Community/Home Visiting \(Acute\) Policy](#)

[RKPG Intake Meeting, Case Allocation and Case Review \(Mental Health\) Policy](#)

[RKPG Mental Health Standardised Clinical Documentation and Process Standard Operating Procedure](#)

[RKPG Mental Health Consumers who Do Not Attend Community Appointments: Management of \(Mental Health\) Policy and Procedure](#)

[RKPG Mental Health Service Client Identification in the Community](#)

[SMHS Care coordination in mental health](#)

National Standards



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